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February 5, 2025

By E-mail: bospositionstmts@mass.gov; gina.leonard@mass.gov

Gina Leonard, Investigator
c/o Tania Taveras, Administrative Assistant
The Commonwealth of Massachusetts
Commission Against Discrimination
One Ashburton Place
Boston, MA 02108

Re: Stafanie Charity v. Boston Children's Hospital
MCAD Docket No. 24BEM03491
EEOC / HUD No. 16C-2025-00293

Dear Ms. Leonard:

Please accept the following as the position statement of Respondent, The Children's Hospital Corporation, also known as Boston Children's Hospital (hereinafter, "Respondent" or "BCH") in response to the Charge of Discrimination ("Charge") filed by Complainant, Stefanie Charity ("Complainant"). Complainant, a former Dental Hygienist at BCH, alleges that she was subjected to discrimination on the basis of her race (African American) and color. Specifically, Complainant alleges that BCH was subjected to unwarranted criticism and performance counseling regarding the content and untimeliness of the clinical notes that hygienists are required to prepare after each encounter with a pediatric patient. She disputes that her clinical notes were inadequate, and she claims without evidence that she was held to a different standard than other hygienists. Complainant voluntarily resigned her position, less than a year after she was hired, based on her belief that she was subjected to unfair criticism of her performance.

BCH denies these allegations and submits that Complainant was not subjected to discrimination of any kind. The documented record in this case demonstrates that Complainant's clinical notes were consistently incomplete, inaccurate, and submitted in an untimely way. Her supervisors worked with her extensively to point out these deficiencies, and they spent a lot of time and effort to help her succeed, including regular meetings. Complainant failed to meaningfully acknowledge any deficiencies, and she failed to improve her performance, which led to further performance counseling. Complainant did not have any prior experience working with pediatric patients who often came to the dental clinic with complicated medical situations and histories, and she also lacked experience working in a hospital setting, and thus she failed to accept or understand the importance of detailed, accurate and timely clinical notes.

Complainant voluntarily resigned her employment in apparent protest over what she perceived to be unfair criticism of her performance. To the extent that Complainant claims that she was subjected to greater scrutiny and unfair criticism of her clinical notes relative to her peers, and claims that this was due to her race, this claim is belied by her own admission in the Charge that one of her peers was also African American, but was not counseled for her

performance. Moreover, the record in this case demonstrates that there were specific and repeated complaints about her performance by her managers and the dentists with whom she worked.

Since Complainant has not and cannot offer any evidence to support her claim of discrimination, the Commission should issue a finding of no probable cause and promptly dismiss the Charge.

I. FACTUAL BACKGROUND AND ANALYSIS

A. Boston Children's Hospital

BCH is a world-renowned acute care children's hospital located in the Longwood Medical Area in Boston. The hospital is home to the largest hospital-based pediatric research program in the world, treating pediatric patients throughout Massachusetts, the United States, and across the globe.

BCH proudly employs individuals from a wide range of backgrounds and places a strong focus on diversity, equity, and inclusion. BCH has no tolerance for any type of discrimination, retaliation or harassment in the workplace and has equal employment policies that are set forth in BCH's Policy Against Discrimination. **See Exhibit A**. These policies apply to all BCH employees, and BCH is strongly committed to providing a workplace that is free of discrimination or harassment of any kind.

BCH's Code of Conduct sets forth additional rules and expectations with which all employees are expected to comply. Specifically, the Code of Conduct makes clear that communication, respect, excellence, accountability, teamwork and innovation are core values of the organization. The Code of Conduct can be found here: https://www.childrenshospital.org/sites/default/files/2022-03/2022-code-of-conduct.pdf

B. Complainant's Hire / Dental Hygienist Role

Complainant was hired in late September 2023 as a Dental Hygienist in BCH's Dental Services department. Her immediate supervisor was Ashley Vazquez, and her next-level manager was Lindsey Kolinski. Dr. Man Wai Ng is the Chief of the Department of Dentistry at BCH and thus had overall clinical responsibility for the Dental Services department.

As a Dental Hygienist, Complainant was responsible for performing standard dental hygiene treatments, oral evaluations, preventive dental health education, and mouth care for BCH's pediatric patients. After the Dental Hygienist's work is completed, the patient is seen briefly by a licensed dentist at each visit. Many of the pediatric patients in the Dental Services clinic are patients at BCH for other medical issues, and therefore often present with a host of other and sometimes complicated medical backgrounds. In this setting, it is critically important for hygienists to take detailed, accurate and timely clinical notes of each encounter with a

patient. This is important so that the dentists can quickly access accurate information about the patient in connection with ongoing care, discussions with parents, and optimizing subsequent visits, and also so that patients and families can have access to timely and accurate clinical notes.

Complainant was hired at the same time as a number of other hygienists, and she and her peers were all given substantial training so that all would understand BCH's standards and procedures for each part of their jobs, including the preparation of clinical notes. Complainant and some of her peers did not have experience working with pediatric patients with medically complex needs, as they came from dental practices that either focused on adults or children without complex medical histories. As such, the training was particularly important to ensure that they understood the specific requirements at BCH. In addition, Complainant and others were given additional time to get up to speed, as their managers understood that there was a learning curve for getting up to speed with the unique requirements at BCH.

C. Complainant's Failure to Prepare Complete, Accurate and Timely Clinical Notes

In her Charge, Complainant takes issue with the fact that she was repeatedly issued verbal and written counseling due to her failure to prepare clinical notes that were complete, accurate, and timely. Tellingly, she notes that she met with Dr. Ng in January 2024, only a few months after her hire, who informed her that her clinical notes were not being completed correctly, and Complainant states that she "believe[d] she was correctly completing notes." (Charge, ¶4). This statement is illustrative of the challenges that Complainant's managers had with her throughout her brief tenure; she failed or refused to understand or acknowledge that she was not preparing clinical notes properly, despite far more experienced professionals explaining this to her. She offers no basis in her Charge for her conclusion that she was completing them correctly, much as she offered no basis – other than unsupported disagreement – to her managers at repeated counseling meetings on this topic.

As noted above, it is critical for dental hygienists to take complete and accurate clinical notes, and to enter them into BCH's computer system in a timely fashion. Hygienists were given time every week to perform this important task, and Complainant's peers had no problem doing so. By contrast, Complainant's notes were often inaccurate and incomplete, as she would submit cursory notes that failed to fully or accurately present the details of a visit with a patient, or the dentist's observations and comments about the patient's condition and/or future care.

Complainant's managers observed these issues when reviewing her notes, and they also received complaints from residents and attending dentists about the deficiencies in Complainant's clinical notes. For example, one attending dentist complained in May that Complainant claimed that her "notes disappeared" when she received notes from Complainant a month late which were blank. The attending dentist also pointed out that she would have to resort to completing findings herself right after a patient encounter, rather than waiting for

Complainant's notes (which would be the usual process) because they were often late and/or inaccurate. **See Exhibit A**.

On another occasion, an attending dentist reported to Dr. Ng his own frustrations with Complainant's work, and he pointed out that several residents had gone out of their way to give her constructive feedback as well. He pointed out that Complainant was often dismissive and not receptive to this feedback, rather than accepting it in a constructive way. **See Exhibit B**.

In May, an attending dentist emailed Complainant to detail specific concerns she had with a clinical note she had prepared. The dentist noted that the problems with the clinical note meant that they would have to bring the patient back in again for a consult to determine a treatment plan for the patient, impacting the patient's ability to receive timely care for a particular tooth, and impacting patient satisfaction. In typical fashion, Complainant responded with defensiveness rather than acknowledgement of the issues and a commitment to improve. **See Exhibit C**.

Complainant would also repeatedly fail to submit her notes in a timely way, and in some cases, she would fail to submit them at all. On many occasions, she would not submit her notes of a given patient encounter for many weeks after the encounter, when it would have been nearly impossible for her to remember all of the details of a visit. In addition, Complainant appeared to be uncomfortable with using electronic medical records software, as she typically took inadequate handwritten notes of these visits, which she would then have to enter into the computer system, thus exacerbating this issue. After being counseled about this issue repeatedly as well, Complainant's timeliness in submitting clinical notes improved somewhat, but the notes frequently continued to be incomplete or inaccurate.

1. Complainant's Managers Meet With Her to Discuss Deficiencies with Her Clinical Notes

As noted above, Complainant and her peers were given ample time and training to get up to speed on each of the aspects of their jobs, including the preparation of clinical notes. Complainant's peers demonstrated proper preparation of clinical notes, and Complainant did not. Her managers, including Ms. Vazquez, Ms. Kolinski, and Dr. Ng, spent a lot of time and effort speaking with and meeting with Complainant to help her to understand the problems with her clinical notes so that she could succeed. However, these deficiencies continued, and by April 2024, they felt the need to establish more formal meetings to address these issues and set expectations.

On April 10, 2024, Dr. Ng, Ms. Vazquez, and Ms. Kolinski met with her to review the concerns with her clinical notes, including the quality of the notes and the untimeliness. They also pointed out that she was often using inappropriate and unprofessional verbiage in her notes, and that she was failing to use the proper dental charting software to document clinical findings, diagnoses, and treatment plans. They conveyed specific expectations going forward, which

included that clinical notes had to be completed and routed to the examining dentist within five days of the date of service, so that the dentist could review and sign off on the note, consistent with standard BCH practice. They also reiterated that Complainant's clinical notes must be complete, of high quality with necessary details included, and written using clinical, factual verbiage to accurately document clinical findings and treatment rendered. The details of this meeting were set forth in notes intended to memorialize these discussions. **See Exhibit D**.

A week later, Complainant was shadowed by Dr. Ng so that she could be given direct feedback from Dr. Ng as to how she was performing. Dr. Ng noted ongoing concerns with Complainant's handoffs to the examining dentist, finding that her reporting and initial documentation was unprofessional and not sufficiently descriptive. Dr. Ng debriefed on this shadowing session with Complainant, and it was memorialized in writing as well. *See Exhibit D.*

Complainant's managers met with her again on May 29, 2024, noting that while the timeliness of her notes completion had improved, there were ongoing issues with the quality, accuracy, and completeness of her clinical notes. They reiterated the performance expectations going forward and memorialized them in writing. *See Exhibit D.*

D. Complainant is Issued a Formal Written Counseling

Despite the substantial efforts of Complainant's managers to give her constructive feedback and counseling on the issues with her clinical notes, and the significant time and resources that were spent in helping her to succeed, she failed to improve in a meaningful way. As a result, she was issued a Written Counseling, dated July 25, 2024. This document detailed the ongoing issues with her clinical notes, and noted that there was no significant improvement since the prior coaching sessions noted above. The document noted that Complainant's performance failed to meet BCH's core performance standards in multiple respects, and it proceeded to set forth specific performance expectations going forward. These included that her signed clinical notes must be submitted within five days of a patient visit, and that they must be complete, accurate and contain sufficient detail to document the visit. There were additional expectations set forth as well, and the document stated that if she failed to meet these standards, further disciplinary action will result. Complainant refused to sign this Written Counseling document. **See Exhibit E.**

Despite the ongoing performance counseling, Complainant's performance did not improve in the ensuing weeks. For example, on July 24, a Patient Revenue Manager had to email her to point out that two patients who were seen by Complainant a full month earlier still had no clinical notes in the system, which would also impact BCH's ability to bill for those visits. **See Exhibit F**.

E. Complainant Resigns From Her Position

On August 25, 2024, Complainant sent an email to a large group of people at BCH abruptly announcing her resignation, including her peers in Dental Services, dental residents, dentists, and department managers. She complained about the criticism she had received for her deficient clinical notes, and even attached copies of the performance counseling notices she had received during her tenure. In typical fashion, Complainant refused to take any responsibility for her deficiencies, and again claimed – without any evidence whatsoever – that she was being subjected to different standards with respect to clinical notes. **See Exhibit G**.

II. ANALYSIS

Complainant's claim of race discrimination is entirely without merit, both as a matter of fact and law. As a preliminary matter, she was hired by the same managers who were fully aware of her race at the time of her hire, and who were the ones who issued performance counseling and criticism to her. Why would these individuals hire Complainant, and then immediately proceed to target her – and only her – for deficient clinical notes? Complainant offers no evidence, nor can she, that would answer this critical question.

In addition, Complainant's claim of race discrimination is belied by the fact that she claims that the other dental hygienists were treated more favorably because they were not subjected to performance counseling or criticism. However, one of the hygienists to whom she compares herself is also African American, and therefore her claim that she was singled out due to her race is entirely without merit. (Charge, ¶13).

Complainant's performance deficiencies were well documented, and she was given numerous chances over a period of many months to help her improve her performance. Her managers truly wanted her to succeed, and they expended substantial time supporting her, but she failed or refused to accept any advice or constructive feedback about her clinical notes – despite the fact that she had no prior experience with pediatric patients in a hospital setting, and despite that she was given substantial training and counseling on what was required for the preparation of proper clinical notes. Her deficiencies were observed and reported by the dentists and residents with whom she worked, and also by her managers, and these deficiencies are also well documented. Accordingly, Complainant cannot demonstrate that she was singled out for unjustified criticism. To the contrary, there is a substantial record of these deficiencies, and Complainant cannot offer any evidence to dispute this record.[1]

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[1] Complainant complains that her laptop was often defective. (Charge, ¶3). Respondent submits that this is another example of Complainant seeking to deflect blame for her own deficiencies. While there were minor issues with her laptop, BCH's IT department worked with her to resolve those. While this seems to be a non-material issue, BCH will be happy to submit documentation of this issue should the Commission so request.

Finally, Complainant cannot show that she was subjected to an adverse employment action in terms of the termination of her employment, because she voluntarily resigned. She will apparently contend that she was compelled to resign because of what she felt was unjustified criticism, but the documented record in this case demonstrates that this was not the case, and that her decision to voluntarily resign was entirely her own.

III. CONCLUSION

Complainant has offered no evidence to support her claim that she was subjected to discrimination on the basis of her race. To the contrary, she was treated fairly and on the same basis as her peers, and all of the decisions at issue in this case were supported by legitimate, non-discriminatory reasons. For all of these reasons, BCH respectfully submits that the Charge should be promptly dismissed for lack of probable cause.

Should the Commission have any questions or need additional information to assist in its investigation, please do not hesitate to contact me.

Very truly yours,

Gregory A. Manousos

GAM/gmf
Enclosures
cc: Stefanie Charity [via mail: 4000 Chapel View Boulevard, #1077, Cranston, RI 02920.

AFFIRMATION

I hereby state that with respect to the facts in the foregoing position statement of which I have personal knowledge, I affirm that such information is true and correct. With respect to the facts in the foregoing position statement of which I do not have direct personal knowledge, I affirm, to the best of my knowledge, that such information is true and correct.

Signed under the pains and penalties of perjury this 5 day of February, 2025.

Scott Shannon

Scott Shannon
Lead Employee Relations Consultant
Boston Children's Hospital

EXHIBIT A

From: Choi, Margaret <Margaret.Choi@childrens.harvard.edu>
Sent: Wednesday, May 15, 2024 4:43 PM
To: Yi, Young <Young.Yi@childrens.harvard.edu>; Ng, Manwai <Manwai.Ng@childrens.harvard.edu>; Ang, Jonathan <Jonathan.Ang@childrens.harvard.edu>
Subject: RE: Stefani

Thinking further back - I have encountered a few of Dr. Yi's comments as well, which I previously brought to Lindsey and Ashley's attention (1-2 months ago).

1. I agree – I have heard the "Notes disappeared" explanation when I received notes directly from Med Rec over a month late and was blank.
2. She was unable to discern primary incisors from permanent incisors for Katherine Ezeh (last week). #24 and 25 clearly had mamelons, slightly discolored, and were larger.
3. I have made a habit of writing findings myself in a template as soon as I come back to my computer or in Dentrix notes, since information is not correctly logged and by the time I receive the note a few weeks later I do not remember what or who I saw.

Hope this helps!

Margaret

From: Yi, Young <Young.Yi@childrens.harvard.edu>
Sent: Wednesday, May 15, 2024 4:32 PM
To: Choi, Margaret <Margaret.Choi@childrens.harvard.edu>; Ng, Manwai <Manwai.Ng@childrens.harvard.edu>; Ang, Jonathan <Jonathan.Ang@childrens.harvard.edu>
Subject: Re: Stefani

Dear Dr. Ng,

Unfortunately, Stefani has not shown significant improvement. Critical information is often missing from her notes, and essential details are at risk of being overlooked. When discussing clinical findings, treatment plans, and growth and development, she lacks a solid understanding of these topics, which is concerning.

Additionally, Stefani has repeatedly claimed her notes have disappeared, responding with what appears to be a copied and pasted explanation. This has happened multiple times, indicating a possible lack of accountability or understanding of her documentation process.

Despite our ongoing efforts to train and support her, progress has been limited. This has led to increased workloads for other hygienists, who often need to cover for her.

On a positive note, Stefani does connect well with certain families. However, given the pace and complexity of our patient population, I am concerned this environment may not be the best for her professional growth.

Best regards,

--
Young S. Yi, DMD, FAAPD
Pediatric Dentist, Boston Children's Hospital
Instructor, Harvard School of Dental Medicine

From: Choi, Margaret <Margaret.Choi@childrens.harvard.edu>
Sent: Wednesday, May 15, 2024 4:28 PM
To: Ng, Manwai <Manwai.Ng@childrens.harvard.edu>; Ang, Jonathan <Jonathan.Ang@childrens.harvard.edu>; Yi, Young <Young.Yi@childrens.harvard.edu>
Subject: RE: Stefani

Hi Dr. Ng,

I've been in HUN lately but when I was at BP last week, the day went pretty well.

Looking through the charts from her column last week, I see that she sent her notes to Bryan Shin same day. I have not yet received notes from Bryan so I see some areas that remain incomplete (e.g. radiographic findings that Bryan can interpret and add in), or occlusion.

Parts of her notes are still in informal language – Such as "Sister said they didn't get text reminder for today" as her dental no show note –or- "do nothing restoratively" on radiographic findings. I assume Bryan said something like - "there's small cavities that we don't need to restore". Although thinking of it, Bryan tends to write in informal lingo too.. Interested to see what comes through – hopefully in a timely manner.

At least there are no blank notes and they were all sent out to the resident same day.

Overall I believe there is improvement!

I'll keep feedbacks in mind going forward.

Thank you,
Margaret

From: Ng, Manwai <Manwai.Ng@childrens.harvard.edu>
Sent: Wednesday, May 15, 2024 3:16 PM
To: Ang, Jonathan <Jonathan.Ang@childrens.harvard.edu>; Choi, Margaret <Margaret.Choi@childrens.harvard.edu>; Yi, Young <Young.Yi@childrens.harvard.edu>
Subject: Stefani

Hi Jon, Margaret and Young,
Could you provide some feedback on how Stefani Charity is doing? Especially with respect to her notes?

Thank you, Man Wai

EXHIBIT B

From: Ang, Jonathan <Jonathan.Ang@childrens.harvard.edu>
Sent: Thursday, May 16, 2024 10:54 AM
To: Yi, Young <Young.Yi@childrens.harvard.edu>; Ng, Manwai <Manwai.Ng@childrens.harvard.edu>; Choi, Margaret <Margaret.Choi@childrens.harvard.edu>
Subject: Re: Stefani

Dear Dr. Ng,

I can confirm that I have shared feedback with her. Additionally, I have overheard that several residents have gone out of their way to give her constructive feedback as well. I think the problem that they run into that I also share is that she is oftentimes dismissive and not receptive of things that she can do to improve as a hygienist.

I am disappointed to know that she has declined the opportunity to shadow other hygienists. I do believe that there are very valuable things that she would be able to take away from observing other hygienists including but not limited to proper patient interactions as well as accurate documentation.

One idea that I had in terms of further training would be putting her on a limited hygiene schedule where she saw a patient every other half hour. While I am not sure how feasible that would be with our current clinic flow, I believe it would allow her to hopefully work on those weaknesses of accurate documentation. My expectation with slowing her schedule down would be that in the half hours that she has off, she would be required to provide accurate documentation, treatment plan, and note that requires minimal to no alterations by the supervising dentist. If she is forced to finish her notes immediately after the appointment, I believe this will give us a chance to review the patient with her right away so that there can be no misunderstandings. I can only hope that she would be more receptive to immediate feedback. If she is able to accomplish this, we can then transition her back into her current schedule where she takes care of documentation on her own down time/outside of clinic hours.

Regarding the other hygienists, I will send you my feedback soon.

Sincerely,
Jonathan

From: Yi, Young <Young.Yi@childrens.harvard.edu>
Sent: Wednesday, May 15, 2024 10:51 PM
To: Ng, Manwai <Manwai.Ng@childrens.harvard.edu>; Ang, Jonathan <Jonathan.Ang@childrens.harvard.edu>; Choi, Margaret <Margaret.Choi@childrens.harvard.edu>
Subject: Re: Stefani

Dear Dr. Ng,

I can confirm that I have shared feedback with her as you mentioned.

I am disappointed to hear that she has declined opportunities to shadow other hygienists. I believe additional training is necessary, though I am uncertain of its effectiveness and her willingness to learn.

If we decide to provide further training, it may need to be framed as remediation to engage her. It will need to be well-structured with specific tasks, and we would have to clearly state the department's expectations. Hands-on sessions with a dentist providing direct supervision might help address the gaps in her knowledge and improve her clinical skills. Providing immediate feedback, especially in the beginning, would be crucial to reinforce learning. For instance, she could see patients in the morning under direct supervision of a dentist, then write notes and review charts together in the afternoon. However, I am unsure if this is feasible with our current schedule.

I will send you my feedback on other hygienists before the weekend.

Best regards,

Young

Young Yi, DMD FAAPD

From: Ng, Manwai <Manwai.Ng@childrens.harvard.edu>
Sent: Wednesday, May 15, 2024 10:28:34 PM
To: Ang, Jonathan <Jonathan.Ang@childrens.harvard.edu>; Yi, Young <Young.Yi@childrens.harvard.edu>; Choi, Margaret <Margaret.Choi@childrens.harvard.edu>
Subject: RE: Stefani

Dear Jon, Margaret and Young,
I very much appreciate your thoughtful and detailed responses.
I know many of us have shared feedback with her.
Will you confirm that you also have?

Well written notes and problematic notes have been shared with her.
We have offered, but she has declined opportunities to shadow our other hygienists.
Do you think more training will be helpful for Stefanie? What type of training?

When you have time, will you share feedback on our other hygienists?
Any concerns regarding their notes, charting, etc.?
No need to respond right away about the others.

Thank you again, Man Wai

From: Ang, Jonathan <Jonathan.Ang@childrens.harvard.edu>
Sent: Wednesday, May 15, 2024 5:04 PM
To: Yi, Young <Young.Yi@childrens.harvard.edu>; Choi, Margaret <Margaret.Choi@childrens.harvard.edu>; Ng, Manwai <Manwai.Ng@childrens.harvard.edu>
Subject: RE: Stefani

Dear Dr. Ng,

I share many of the same sentiments as both Margaret and Young regarding Stefani. One thing that has improved with her is that she is actually submitting her notes in a timely fashion. I know in the past there have been several missing notes that I had to personally attend to. However, these cases have become more sparse over the last few months.

However, I do often find her notes missing a lot of information. Having personally done many of her hygiene checks, I have become accustomed to the fact that a lot of what I am saying during my checks is not documented. Additionally, the majority of the charts that I have reviewed do not have an updated odontogram. None of the teeth (primary vs permanent) are documented accurately, no caries/conditions charted, etc. Due to this, I often find myself having to document what I dictated during my checks independently because I no longer have the trust that things will be done properly on her end. Finally, I believe this lack of trust in Stefanie, even amongst the residents, has interfered with clinic flow as residents often try to avoid doing her checks knowing that there will be more additional work put on their shoulders as compared to the hygiene check of another hygienist.

Additionally, I echo what Margaret has said regarding the verbiage of her notes. Often times the notes are written in a very unprofessional manner and would not be something I would be comfortable having patients' parents read. I think this also carries over with how she sometimes treats patients who happen to be a little bit more difficult behavior wise.

Happy to elaborate on any of these comments and hope this helps.

Best,
Jonathan

From: Yi, Young <Young.Yi@childrens.harvard.edu>
Sent: Wednesday, May 15, 2024 4:32 PM
To: Choi, Margaret <Margaret.Choi@childrens.harvard.edu>; Ng, Manwai <Manwai.Ng@childrens.harvard.edu>; Ang, Jonathan <Jonathan.Ang@childrens.harvard.edu>
Subject: Re: Stefani

Dear Dr. Ng,

Unfortunately, Stefani has not shown significant improvement. Critical information is often missing from her notes, and essential details are at risk of being overlooked. When discussing clinical findings, treatment plans, and growth and development, she lacks a solid understanding of these topics, which is concerning.

Additionally, Stefani has repeatedly claimed her notes have disappeared, responding with what appears to be a copied and pasted explanation. This has happened multiple times, indicating a possible lack of accountability or understanding of her documentation process.

Despite our ongoing efforts to train and support her, progress has been limited. This has led to increased workloads for other hygienists, who often need to cover for her.

On a positive note, Stefani does connect well with certain families. However, given the pace and complexity of our patient population, I am concerned this environment may not be the best for her professional growth.

Best regards,

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Instructor, Harvard School of Dental Medicine

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Looking through the charts from her column last week, I see that she sent her notes to Bryan Shin same day. I have not yet received notes from Bryan so I see some areas that remain incomplete (e.g. radiographic findings that Bryan can interpret and add in), or occlusion.

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At least there are no blank notes and they were all sent out to the resident same day.

Overall I believe there is improvement!

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Margaret

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Subject: Stefani

Hi Jon, Margaret and Young,
Could you provide some feedback on how Stefani Charity is doing? Especially with respect to her notes?

Thank you, Man Wai

EXHIBIT C

From:	Charity, Stefanie
To:	Steff
Cc:	Vazquez, Ashley
Subject:	Fwd: Feedback for note for MRN 4657089
Date:	Friday, May 31, 2024 8:30:55 AM
Attachments:	Outlook-Boston Chi.png Outlook-Boston Chi.png

From: Charity, Stefanie <Stefanie.Charity@childrens.harvard.edu>
Sent: Friday, May 31, 2024 8:00 AM
To: Smyth, Elaine <Elaine.Smyth@childrens.harvard.edu>
Subject: Re: Feedback for note for MRN 4657089

Hello Dr Smyth,

That email you sent was yesterday at
10:25 am approximately.
I didn't see it since we were in Clinic, otherwise I would have responded back even sooner.

It's 7:10 am and I'm bothered that this patient did not receive the Referral as needed.

When you say the Referral wasn't in the Chart, do you mean a Scanned Referral document?
Usually when a Provider is given a Referral, we give it to them in that approintment.

Im not on Campus but will clock in and prioritize this note.

As you can see by the last email response to you ; was 7:07 pm last night as I was catching up with Notes last night on Campus.

Re: this patient, if I took a PA, Bwx or Pan , I have the Xrays displayed bwx, PAs, and Romexis up while waiting for the Patient.
Then, we plan a Tx plan.
Not sure why this Exam didn't occur in this manner.
If I put OMFS referral, perhaps b/c we weren't sure if or what the patient would need.
More than likely I put that in Dentrix and not Power Chart since the Note may have not been completed but I wanted you to have that piece of information.

The times that you and I have worked together, you have personally brought a Referral and the patient is walked out.

Not sure when you are on Campus.
But I will be completing Notes today.
If you're there on Monday, we can review this note.
I will pull up the MRN now and get the patient name.
If you are on Campus, my cell is
617 823 9757,

Stefanie

From: Charity, Stefanie <Stefanie.Charity@childrens.harvard.edu>
Sent: Thursday, May 30, 2024 7:40:47 PM
To: Smyth, Elaine <Elaine.Smyth@childrens.harvard.edu>
Subject: Re: Feedback for note for MRN 4657089

I have left Campus and re read your email.
This is very concerning.

From: Charity, Stefanie <Stefanie.Charity@childrens.harvard.edu>
Sent: Thursday, May 30, 2024 7:07:41 PM
To: Smyth, Elaine <Elaine.Smyth@childrens.harvard.edu>
Subject: Re: Feedback for note for MRN 4657089

Re: I'm just getting this note tonight at 7:07 pm.

From: Smyth, Elaine <Elaine.Smyth@childrens.harvard.edu>
Sent: Thursday, May 30, 2024 10:25 AM
To: Charity, Stefanie <Stefanie.Charity@childrens.harvard.edu>
Cc: Vazquez, Ashley <Ashley.Vazquez@childrens.harvard.edu>
Subject: Feedback for note for MRN 4657089

Hi Stefanie,

Hope you are well. I wanted to reach out to review this note with you - I have added to the note and finalized it, it may be helpful for you to review it and see the changes that were needed (my amendments are initialed).

I did have some concerns with the note that I would like to share:

- A PA of #3 was taken as well as a PANO, but no findings were recorded for these images in the note.
- The PANO was also not imported into Dexis.
- The next visit was listed as "restorative" - he needs 3-4 restorative visits, it is helpful to outline the number of visits needed and what they may be planned for at the end of the note.
- In this case, tooth #3 has a large caries lesion approximating the pulp. Based on the radiographs, it may need RCT or extraction. The Dentrix notes box states "OMFS referral?" but this is not included in the Powerchart note, and there is no OMFS referral

on file. (My workflow for hygiene patients requiring OMFS referrals is typically to do the referral immediately after the check when I return to the attending office, as I know it can take some time for me to get through all of the notes sent for signing.)

Ultimately, these issues with documentation mean we have to bring this patient back again for a consult to determine a clear plan for this tooth. This may impact this patient's ability to receive timely care for this tooth, in addition to impacting their satisfaction with the care they receive and also their trust in our team at BCH Dentistry.

I am always very happy for you to seek clarification from me, either in person at the time or later via email, if there is anything unclear from a hygiene check - we are all working together to ensure that our patients receive the care they need!

Thank you for taking the time to consider this email and to review the note.

Best,
Elaine

Dr. Elaine Smyth BA, BDentSc, MFDS (RCSI), DChDent
Attending in Pediatric Dentistry
Department of Dentistry
Boston Children's Hospital
300 Longwood Avenue, Boston, MA 02115
tel:617-355-6571 | fax:617-730-0478

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EXHIBIT D

Employee name: Stefanie Charity

ID #: 251896

Job Title: Dental Hygienist

Supervisor: Ashley Vazquez

Hire Date: 09/25/2023

Previous meetings, performance concerns, and performance expectations previously established:

04/10/2024 – Stefanie, Ashley, Lindsey

- Performance concerns
  - Clinical progress note quality, completeness, and timeliness on completion are not meeting previously established expectations.
  - Dental hard tissue charting is not being completed within Dentrix, our dental charting software, to document clinical findings, diagnoses, and treatment plans communicated from the dentist during dental examinations.
  - Inappropriate and unprofessional verbiage is being utilized within the clinical note documentation that is not demonstrative of clinical findings.
- Performance expectations
  - Initial iteration of clinical progress note completed by Dental Hygienist is expected to be signed and routed to examining dentist within 5 days of the date of service.
  - The Dental Hygienist's signed note must be complete, of high quality with necessary details included, and written using clinical, factual verbiage to accurately document clinical findings and treatment rendered.

04/17/2024 – Dr. Ng shadowing Stefanie Charity AM, debrief thereafter – Stefanie, Ashley, Dr. Ng

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- Performance concerns
  - Handoffs / reporting out to examining dentist after prophylaxis and initial documentation are unprofessional and non-descriptive in nature. The verbiage used to translate patient and appointment information to the dentist does not accurately describe clinical findings in detail to the examining dentist
  - Managing scheduled appointment volume efficiently through critical thinking and time management skills
- Performance expectations
  - Verbal communication between dental hygienist and examining dentist are expected at every exam for every patient including information on the patient's medical and dental history, clinical findings, and treatment rendered in professional, clinical, and factual verbiage that is pertinent to the appointment.
  - Every Dental Hygienist is expected to see 10 – 15 patients per day
  - All aspects of the dental recall appointment should be completed within the scheduled visit time allotted as follows:
    - Toothbrush and flossing instructions: ~ 5 minutes
    - Coronal polishing with prophy angle: ~ 5 minutes
    - Capturing radiographs: 5 – 10 minutes
    - Exam: 5 – 10 minutes
    - Turning over operatory: ~ 5 minutes
  - In the case of IT / computer challenges, Stefanie is expected to complete administrative hours (for notes completion, future appointment review, etc.) on site on BCH's desktop computers rather than remotely.

05/29/2024 – Stefanie, Ashley, Lindsey, Dr. Ng

- Performance concerns
  - While timeliness on notes completion has improved, clinical progress note quality, accuracy, appropriate verbiage, and completeness are not meeting previously established expectations.
  - Dental hard tissue charting is not being consistently completed in Dentrix (as discussed in previous meetings); thus, clinical documentation is not meeting expectations.
- Performance expectations
  - The Dental Hygienist's signed note must be complete (within 5 days), of high quality with necessary details included, and written using clinical, factual verbiage to accurately document clinical findings and treatment rendered.
  - Clinical findings and treatment plans verbally transferred to dental hygienist from examining dentist are to be consistently documented by dental hygienist in clinical progress note.

EXHIBIT E

Boston Children's Hospital

Where the world comes for answers

EMPLOYEE CORRECTIVE ACTION NOTICE: WRITTEN COUNSELING

Date: 07/25/2024
Employee Name: Stefanie Charity ID#: 251896
Job Title: Dental Hygienist
Department: Dentistry
Supervisor: Ashley Vazquez
Hire Date: 09/25/2023
RE: Performance Expectations

Stefanie Charity, this counseling will summarize our meeting on 07/25/2024.

We discussed the recent concerns with your performance as it relates to:

- No noted improvement sustained in repeated behaviors since verbal coaching (indicated below)
- Clinical progress note quality, completeness, and timeliness on completion
- Dental hard tissue charting, documentation of clinical findings, diagnoses, and treatment plans
- Inappropriate and unprofessional verbiage within the clinical note that is not demonstrative of clinical findings.
- Handoffs / reporting out to examining dentist during exam
- Managing scheduled appointment volume
- We discussed the importance of correct and appropriate clinical documentation, as erroneous documentation is a potential safety risk to Boston Children's Hospital's patients.

On 04/10/2024, 04/16/2024, and 05/29/2024, a documented verbal coaching was discussed with you regarding:

- Clinical progress note quality, completeness, and timeliness on completion
- Dental hard tissue charting is not being completed within Epic (previously in Dentrix) to document clinical findings, diagnoses, and treatment plans communicated from the dentist during dental examinations
- Inappropriate and unprofessional verbiage is being utilized within the clinical note documentation that is not demonstrative of clinical findings.
- Handoffs / reporting out to examining dentist during exam and initial documentation are unprofessional and non-descriptive in nature. The verbiage used to translate patient and appointment information to the dentist does not accurately describe clinical findings in detail to the examining dentist
- Managing scheduled appointment volume efficiently through critical thinking and time management skills

This failure to meet consistent productivity expectations does not meet our BCH Core Performance Standards reflecting:

Accountability: "We hold ourselves accountable to standards of performance. We take responsibility for what we do and we always keep the long-term good of the organization in mind."

Excellence: "We strive for excellence both in what we do but how we do it. We challenge ourselves to continually look for ways to improve what we do in service of patients and families."

Communication: "We communicate clearly, honestly, listen carefully and with an open mind. We respond thoroughly and promptly. We try to be proactive in our communication and willing to surface difficult issues."

Teamwork: "We work with others effectively and rely on the team's support and trust. We acknowledge and demonstrate positive attitudes about the team, its members, and its ability to get its mission accomplished."

During our meeting, we reviewed the following expectations of your role as Dental Hygienist in the Department of Dentistry. Additional performance expectations reviewed included:

- The Dental Hygienist's signed note must be complete (within 5 days), of high quality with necessary details included, and written using clinical, factual verbiage to accurately document clinical findings and treatment rendered.
- Clinical findings and treatment plans verbally transferred to dental hygienist from examining dentist are to be consistently documented by dental hygienist in clinical progress note.
- Verbal communication ('handoff') between dental hygienist and examining dentist are expected at every exam for every patient including information on the patient's medical and dental history, clinical findings, and treatment rendered in professional, clinical, and factual verbiage that is pertinent to the appointment.
- All aspects of the dental recall appointment should be completed within the scheduled visit time allotted, estimated as follows:
    - Toothbrush and flossing instructions: ~ 5 minutes
    - Coronal polishing with prophy angle: ~ 5 minutes
    - Capturing radiographs: 5 – 10 minutes
    - Exam: 5 – 10 minutes
    - Turning over operatory: ~ 5 minutes
- In the case of IT / computer challenges, you're expected to complete administrative hours (for notes completion, future appointment review, etc.) on site on BCH's desktop computers rather than remotely.

Immediate and sustained improvement in your performance is expected. If you fail to meet these performance standards further disciplinary action will result in a Performance Improvement Notice/Final Warning (PIN). Additionally, any other future job performance problems, even those unrelated to the issues above, may also result in a PIN or termination depending on the severity of the situation.

Please sign below to indicate your understanding about the need to improve and willingness to meet the expectations outlined in this corrective action. Please be aware that should you chose not to sign this document, you are still expected to comply with the outlined performance standards.

Employee Signature	Date 07/31/2024
Supervisor Signature	Date
HR Business Partner Signature	Date 7/31/24
Additional Signature, if needed	Date

If you have any personal problems, which may affect your ability to meet these standards or any other job performance standards you are encouraged to use the Hospital's free and confidential Employee Assistance Program. Counselors can be reached 24 hours a day, 7 days a week at 1-800-648-9557.

EXHIBIT F

From: McClellan, Carol
To: Charity, Stefanie
Cc: Vazquez, Ashley
Subject: June patients
Date: Wednesday, July 24, 2024 8:09:06 AM
Attachments: image001.png
Importance: High

Hi Stefanie,

The following patients have you listed as the hygiene provider . I checked Epic and patients were checked in. There are no charges and no notes on both patients.
We have to figure this out . Both patients are well over the 30 period for missing charges , notes.

MRN 6080102 REDACTED 6/20/24 > originally with Danielle, appt changed to You on DOS pt arrived in epic.

MRN 5216770 REDACTED 6/24/24 > patient arrived in epic.

Thank you,
Carol

Carol McClellan/ Patient Revenue Manager /Department of Dentistry / Boston Childrens Hospital/ 300 Longwood Ave. Mail Stop BCH3473 Boston, MA 02115/ ph 617-355-6910/ fax 617-730-0478
mailto:carol.mcclellan@childrens.harvard.edu

SUPPORT HEALTH EQUITY END RACISM
Boston Children's Hospital

EXHIBIT G

From: Charity, Stefanie <Stefanie.Charity@childrens.harvard.edu>
Sent: Sunday, August 25, 2024 9:35:56 PM
To: Dental Residents-dl <DentalResidents-dl@childrens.harvard.edu>; Harper, Matthew <Matthew.Harper@childrens.harvard.edu>; Lambert, Frederick <Frederick.Lambert@childrens.harvard.edu>; Yi, Young <Young.Yi@childrens.harvard.edu>; Cho, Yun <Yun.Cho@childrens.harvard.edu>; Choi, Margaret <Margaret.Choi@childrens.harvard.edu>; Ang, Jonathan <Jonathan.Ang@childrens.harvard.edu>; Smyth, Elaine <Elaine.Smyth@childrens.harvard.edu>; Sulyanto, Rosalyn <Rosalyn.Sulyanto@childrens.harvard.edu>; Grady, Victoria <Victoria.Grady@childrens.harvard.edu>; Taylor, Danielle <Danielle.Taylor@childrens.harvard.edu>; Doucette, Danielle <Danielle.Doucette@childrens.harvard.edu>; Hallisey, Ellen <Ellen.Hallisey@childrens.harvard.edu>; HR Hub <HRHub@childrens.harvard.edu>; Shannon, Scott <Scott.Shannon@childrens.harvard.edu>; Locke, Leslie <Leslie.Locke@childrens.harvard.edu>; Ng, Manwai <Manwai.Ng@childrens.harvard.edu>; Melo Pimentel, Vivian <Vivian.MeloPimentel@childrens.harvard.edu>; Cancinos, Christopher <Christopher.Cancinos@childrens.harvard.edu>
Subject: What an Experience!

To Whom It May Concern:

Effective Immediately- Resignation

If you receive this email, it means IT has not rescinded it.

I thought Children's would be my "forever" place. The kiddos and their families who express gratitude for what I have done is an Amazing feeling.

Clinical documentation is extremely important as well as subjective!! But if you are critiquing only my Charts and no other Hygienists.. that's a problem.
No time stamp for any other Hygienist but me.
The accusations of "I'm the only Hygienist keeping the patient in the Chair too long" I'm the only Hygienist not completing notes on time" "I'm the only Hygienist who does not stay on Schedule " "that I am writing offensive notes and erroneous notes" and then having weekly meetings about Charts twice a week for 30 minutes is excessive . The 30 min twice a week meetings implemented by Ashley Vasquez who is no longer here..also leaves me speechless

and has culminated in additional unnecessary stress and an uncomfortable work environment.
Not sure why you are singling me out.
These scenarios listed above are some of the reasons I am resigning.
You may feel a sense of power or accomplishment or that you have won. You have not.

It is apparent that Admin has not consulted the Other Hygienists.

To have 6 meetings with me and no Employee Ally, but then present a corrective action document from HR with these accusations insisting I sign is equally disturbing.

Two loaner laptops with chronic issues which A. Vasquez and Leslie have known about went unresolved.

If you consult the IT Department with the log of my WFH time used to resolve issues before Chart documentation could be started let alone completed.

To be told by a Dental Assistant Ashley Vasquez, Hygiene Supervisor on more than one occasion , that she could help me transition out of the BCH.

To have a Hygiene meeting with all hygienists and then be invited to have a 1:1 meeting about Ashley but then that meeting was about Chart documentation. Another blindsided Event towards me.

To have an Annual Merit increase meeting and have it be another Chart documentation meeting session..

To never have any positive feedback from the Chief has contributed to a toxic, hostile, and emotionally distressing experience at my place of work is unacceptable.

I leave this quote with the best advice.

"Leaving a toxic job is an investment in your well being, mental health, and purpose. Staying drains your energy, diminishes your confidence, and stunts your growth. But don't just leave. Learn from your experience and find a healthier work environment where you can thrive, grow, and find fulfillment."

Even my personalized Saddle that I brought in, people found this difficult to respect.
The loaner laptop is in Leslie's office.

Respectfully,
Stefanie Charity, RDH, BS